

Pelvic organ prolapse happens when the support for the pelvic organs weakens. The bladder, the uterus or the rectum then presses into, or out of, the vagina. This causes a **bulge or a feeling of pressure**. Prolapse is very common. It is **not cancer**, and it is rarely dangerous. Good options range from exercises to a support device to surgery.

About This Condition

Prolapse is named for the part of the vagina that drops:

- **Front wall** (the bladder): a cystocele
- **Back wall** (the rectum): a rectocele
- **Top** (the uterus, or the top of the vagina after a hysterectomy)

Common symptoms:

- A **bulge or lump** you can see or feel at the vaginal opening
- Pelvic **pressure or heaviness**, often worse late in the day or after standing a long time
- Trouble fully emptying the bladder or bowel. Some people press on the bulge to go (this is called “splinting”).
- Leaking urine, or discomfort during sex

What Causes It

Anything that weakens pelvic support can play a part: **pregnancy and vaginal birth**, getting older, extra weight, and a family tendency. Straining from constipation or a long-term cough adds to it. Many people have more than one cause.

Learn the Terms

Prolapse

When a pelvic organ drops from its normal place.

Cystocele

The bladder bulging into the front wall of the vagina.

Rectocele

The rectum bulging into the back wall of the vagina.

Vaginal vault

The top of the vagina. It can drop after a hysterectomy.

Pessary

A removable support placed in the vagina to hold organs up.

Splinting

Pressing on the bulge to help empty the bladder or bowel.

Pelvic floor

The muscles that support the pelvic organs.

POP-Q

The exam used to measure how far the prolapse drops.

Do I have to treat it? Only if it bothers you. If you have no symptoms, or only mild ones, you can simply watch it. If it affects your comfort or daily life, you can choose from several good options. There is no single “right” answer, and you can change your mind later.

How It's Diagnosed

- A **pelvic exam**, often while you bear down or stand, to see which parts drop and how far
- Questions about your bladder, bowel and sex life
- Sometimes a check of how well your bladder empties, or **bladder testing** if you also leak urine

How It's Treated (Step by Step)

- 1 **Watching it** if it does not bother you. Treat constipation and manage your weight.
- 2 **Pelvic floor exercises**, ideally with a physical therapist for at least 4 months, for mild prolapse.
- 3 A **pessary**: a removable support that eases the bulge without surgery.
- 4 **Surgery**: a repair that restores support, or a closing of the vagina (colpocleisis) for people who no longer want vaginal sex. Each has its own handout.

Living With Prolapse

- Keep stools soft and **avoid straining**.
- Keep up pelvic floor exercises. Treat a long-term cough.
- A pessary can work for years, not just as a short-term fix.

Call the office that is treating you if you have:

- A bulge that is **painful, bleeds, or looks raw**
- A bulge you can no longer push back in
- Trouble emptying your bladder or bowel

Go to an emergency department if:

- You **cannot urinate** for several hours and your bladder feels full or painful
- You cannot reach the office and are worried

Three Things to Remember

1. Prolapse is common and is not cancer.
2. Treat it only if it bothers you. Options range from exercises to a pessary to surgery.
3. Soft stools and pelvic floor exercises help, whatever you choose.